ALLIANCE OF
WOUND CARE
STAKEHOLDERS
POWER THROUGH COLLECTIVE ADVOCACY

June 16, 2025

Honorable Robert Kennedy
Secretary
Department of Health and Human Services
Centers for Medicare & Medicaid Services
Attention: CMS-0042-NC,
P.O. Box 8013,
Baltimore, MD 21244-8013

Submitted via regulations.gov

Re: **Request for Information; Health Technology Ecosystem**

Dear Secretary Kennedy,

On behalf of Alliance of Wound Care Stakeholders ("Alliance"), Chair Dr. Matthew G. Garoufalis, Vice Chair Kara Couch and I are pleased to provide our recommendations on the Administrations' request for information on the Health Technology Ecosystem. The Alliance is a nonprofit multidisciplinary trade association of physician medical specialty societies, clinical and patient associations, wound care provider groups, wound care clinics and wound care product manufacturers whose mission is to promote quality care and access to products and services for people with wounds through effective advocacy and educational outreach in the regulatory, legislative, and public arenas.[i]

The 21st Century Cures Act of 2016 included several provisions intended to address barriers to electronic data sharing, reduce administrative and cognitive burden associated with certified health information technologies, and hold actors accountable for blocking or inhibiting the flow of patient data throughout the health care system. However, the demand and dependencies on timely and high-quality electronic health information across the health care and technology innovation ecosystems have grown, actors are not being held accountable for blocking or inhibiting the flow of patient data, and the health care delivery system is changing - all pointing to the necessity for change. The Alliance appreciates the focus the Agency is taking to address gaps and challenges in data sharing and our health data infrastructure.

This work is not only essential to empowering patients with a full view of their digital health record and clinicians with the information they need to make informed care decisions, but it also helps to support biomedical innovation, improve public health responses, and can and will modernize coverage**.**

Accelerating the adoption and use of data standards, protecting individuals' privacy, and readying the healthcare ecosystem for adoption and implementation of artificial intelligence tools are all important topics to be addressed in order to address remaining gaps and existing challenges with data sharing. Our comments on the RFI follow.

5225 Pooks Hill Rd | Suite 627S | Bethesda, MD 20814
T 301.530.7846 | **C** 301.802.1410 | **F** 301.530.7946
marcia@woundcarestakeholders.org

General Comments

The Alliance has the following general comments:

1. Medicare insurance and drug programs are difficult to navigate when a Medicare beneficiary is trying to choose a plan that works. Most seniors have a difficult time navigating all of the choices, what they mean and what is the best plan for them. Most seniors are not technologically savvy and cannot do web searches. Even so, many plans are not transparent in the benefits being offered and/or the services covered especially in the managed care space and therefore the searches are often complicated. While this is not a wound care specific issue, it is an area in which we are all impacted and our clinical community often has to address when treating their patients who have been led to believe that their wound care treatments will continue to be covered when, in fact, many plans do not cover or significantly limit the services being provided. We believe that tools need to be created that are transparent, easy to use and accessible to all Medicare beneficiaries. In addition, the plan options need to be more simplistic and easier to understand.
2. Accessing our own health records is often challenging and does not allow for care coordination unless we as patients are the coordinators of information (conduit). Our health records should be accessible to all providers on our care team/journey. It should not be necessary to bring paper copies and multiple discs of our records each and every time we see a new provider in order for them to understand the condition in which we are seeking their care.
3. It appears that electronic health record developers are often the barrier to effective care coordination as well as data collection (quality metrics etc.). This is hugely problematic not only for care coordination but for facility implementation of quality metrics to ensure that patients are receiving the best care possible.
4. Patients are not always very compliant with plans of care. The medical model is moving more care into the patient's home and therefore, monitoring the patient is becoming increasingly important. The Alliance supports implementing a comprehensive digital health ecosystem that integrates telehealth services, remote patient monitoring, and community-based health applications as well as the adoption/coverage of devices that will help to guide a patients' plan of care including Artificial Intelligence (AI). The system should incorporate any and all remote monitoring devices such as blood pressure cuffs, glucose monitors, and pulse oximeters that transmit data directly to healthcare teams for timely interventions as well as point of care imaging technologies for wound care patients (we are not referring to portable x-rays but rather point of care imaging for wound care patients to address the size of the wound as well as bacteria, bioburden, and infection).
5. Managing different workflows, processes, and requirements for different types of plans is becoming increasingly difficult and burdensome. Pushing alignment across various government plans as well as commercial plans would be extremely helpful in many areas including:

   - Interoperability requirements
   - Prior authorization requirements
   - Price transparency requirements
   - Reporting requirements

   There are some requirements that already extend across all or most plan types (i.e. provisions in the No Surprises Act), but many data, interoperability, and health IT requirements imposed by law and subsequent

regulations are only applicable to government plans (one or more of Medicare, Medicaid, CHIP, exchange plans, etc.) and sometimes there are inconsistencies and differences between the various government plans. This makes it challenging and burdensome for providers with patients spanning different types of payer coverage.

Specific Comments

Within the RFI, the Agency posed specific questions. The Alliance has provided, for the Agency's consideration, our responses to those questions below in the areas that we have expertise.

Patients and Caregivers

PC-3: Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?

There are several health management, care navigation, and personal health record (PHR) applications that could be highly beneficial to Medicare beneficiaries and their caregivers, particularly in the context of chronic wound care. Both remote patient monitoring and point of care imaging devices exemplify digital health products supporting individuals and providers across care settings by leveraging AI and technological integration to provide predictive health insights as well as information to help inform plans of care aligning with CMS's goals of improved health outcomes and provider burden reduction. These tools can support better self-management, enhance care coordination, and reduce preventable complications—especially for older adults managing complex conditions like diabetic foot ulcers, venous leg ulcers, or pressure injuries. These apps/tools include the following:

1. Health Management Apps with Wound Care Device Integration

Apps that integrate with advanced wound care devices can help patients and caregivers track healing progress, medication adherence, and signs of complications:

- Point of Care Wound Imaging platforms like Mimosa, Modulim, Spectral, Swift Medical, MolecuLight, or Intellicure use smartphone or tablet cameras, often with 3D capability or AI algorithms, to monitor wound size and tissue health over time. Some allow patients or caregivers to capture wound images at home, which can be remotely assessed by clinicians. These devices with their associated platforms can serve as an extension of care between visits and reduce unnecessary clinic travel for Medicare patients with mobility issues. Some devices can also detect infection and bioburden that are not always seen by the naked eye thus allowing clinicians to establish and/or alter care plans so that the treatment is the most beneficial to the patient.
- These platforms often provide dashboards and alerts, enabling real-time remote monitoring by clinicians—making them valuable tools for home health providers and care navigators managing patients in rural or underserved areas.

2. Care Navigation and Coordination Tools

Apps that support shared care plans and care team communication are particularly important for Medicare beneficiaries, who often receive care from multiple providers.

- Tools like *MyChartCare Companion* can help coordinate wound care schedules (e.g., dressing changes, follow-up imaging, debridement), while also sending reminders for compression therapy or off-loading device use—key adherence factors in wound healing.
- Some apps enable two-way communication between patients, caregivers, and wound care specialists, helping to escalate care early if healing stalls or infections emerge.

Commented [KR1]: Are there any others that I should put in here?

3. Personal Health Record (PHR) Apps

Commented [KR2]: Are there any others that I should put in here?

PHR platforms such as *Apple Health* or *MyChart* enable beneficiaries or their caregivers to store, access, and share wound care-related data across providers:

- These platforms are useful for organizing care notes, wound imaging, medication lists, and treatment milestones.
- Medicare patients frequently transition between care settings (e.g., from hospital to skilled nursing to home health), and PHRs can help ensure continuity—especially when wound care protocols are complex or customized.

In order to promote adoption of these types of apps/tools, the Alliance recommends that CMS could:

- Encourage integration of wound care device data into PHR and care navigation platforms, using standards like FHIR to support interoperability.
- Develop a standard for the shared information as the ability to exchange information is not enough.
- Provide technical and financial incentives to wound care clinics and home health agencies that engage patients using these tools.
- Evaluate the inclusion of digital wound care platforms in Chronic Care Management (CCM) or Remote Therapeutic Monitoring (RTM) billing structures to ensure they are accessible to Medicare populations.

Apps that connect wound care devices with patient self-management and care navigation tools can significantly enhance outcomes for Medicare beneficiaries. These tools empower both patients and caregivers to play an active role in healing, prevent complications, and improve communication across the care continuum.

PC-5: What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

CMS can encourage adoption through clear and comprehensive reimbursement incentives, education, and reimbursement parity for remote patient monitoring as well as point of care imaging devices.

Lack of a clear reimbursement pathways and coverage policies for these types of devices remains a significant barrier to provider adoption and patient access. We believe that CMS's leadership in establishing reimbursement frameworks and their integration into clinical care would greatly accelerate the deployment and utility of predictive digital health tools.

PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

While digital health technologies offer significant benefits, many seniors face challenges in adopting these tools. Many seniors have limited digital literacy, cognitive impairments, and physical disabilities which can hinder their ability to effectively use health technologies. To ensure equitable access, CMS should consider:

- Implementing user-friendly interfaces tailored for seniors
- Providing training programs to enhance digital literacy
- Ensuring availability of alternative support channels, such as telephonic assistance.

Obtaining input from people who actually use and support these tools will improve design, reduce frustration, and increase adoption/use of these technologies.

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both? What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?

The Alliance has identified several areas in which data is valuable but challenges persist in obtaining that data. These include the following:

Data Type	Challenges	Why It's Valuable
Claims Data	Fragmented across insurers; delays in availability. It is also cost prohibitive to access this information for most entities.	Shows full history of services, diagnoses, cost exposure.
Clinical Notes (SOAP, encounter summaries)	Often not released or buried in PDFs; lack of APIs.	Contextualizes diagnoses, clinical decision-making.
Operative Reports	Inconsistent release policies; hard to parse by laypersons.	Critical for follow-up care and second opinions.
Appointment Schedules & Wait Times	Not standardized; inaccessible via open APIs.	Important for planning, reducing no-shows, improving access.
Price Transparency & Out-of-Pocket Estimates	Incomplete, often not personalized.	Central to decision-making for elective care.

It is important to note that linking clinical data to claims via the tokenization process is prohibitively expensive for most entities (other than pharmaceutical companies). Unbiased clinicians and researchers who want to use such data to understand the cost effectiveness of therapeutics cannot afford to do it. Even without tokenization, simply accessing claims data is cost prohibitive due to the burdensome regulations around access to de-identified data (e.g., an IRB approved protocol and other requirements/limitations).

PC-8: What are specific sources, other than claims and clinical data, that would be of highest value, and why?

The Alliance believes there is high-value data beyond claims and clinical records which can inform patient care and compliance to the plan of care. This includes:

- **Social Determinants of Health (SDOH):**
  - Housing instability, food insecurity, transportation gaps. This data is valuable as SDOH are strong predictors of health outcomes and are critical for risk stratification and care planning.
- **Patient-Reported Outcomes (PROs):**
  - Pain levels, fatigue, mobility, patient satisfaction are also valuable as this information complements clinical indicators and improves patient-centered care.

PC-8: What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

CMS should end the program of EHR certification. The testing protocols are sufficiently divorced from the functions that they are supposed to test that many "certified" EHRs cannot perform those functions. The US Wound Registry (USWR), a CMS endorsed QCDR, attempted to do basic quality reporting for 32 different (mostly podiatry) EHRs and 31 of them could not transmit quality data in the required format. Indeed, none of them could even produce a Continuity of Care Document (CCD). Thus, the certification process is a farce. The process of de-certifying an EHR is so burdensome that it is not worth reporting these deficiencies. Technical certification should not be handled via the regulatory process. The market can and should manage this.

Providers

PR-1: What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients

The Alliance commends any effort which encourages all providers, including those in rural areas, to leverage approved digital health products. In general, current obstacles in utilizing digital health products include lack of reimbursement pathways, limited integration with existing EHR systems, inconsistent interoperability standards across varied data formats, and provider burden related to documentation and workflow disruptions. Rural providers also face infrastructure and staffing limitations.

To encourage broader adoption of approved digital health products, CMS and its partners must address the practical, financial, and infrastructure-related barriers that limit access, especially in rural settings. However, even when approved, the implementation, coverage and payment for these technologies is often a barrier to use. Several technologies are available in the wound care space which could be helpful in patient care but access is limited as a result of implementation, coverage and payment barriers. These products include point of care imaging devices and remote patient monitoring devices when used along products such as negative pressure wound therapy.

While the RFI mentions blood pressure cuff and glucose monitoring (which are also used when treating wound care patients), point of care imaging for wound care is just as important as it allows not only the measurement of the wound to see if there is wound healing progression, but it allows for bioburden, infection and/or bacteria to be identified (which it cannot always by the naked eye). This helps with establishing an appropriate treatment protocol (as a clinician would not put a cellular product on a wound that has an infection as the wound will not heal) as well as helps to ensure that the patients wound is healing. Despite the valuable information that

Commented [KR3]: We need more information for this section or to double check that it is all correct and stated concisely

these devices can provide to the clinician in order to impact patient care plans and wound healing, its use is limited as a result of poor payment and coverage.

Both remote monitoring as well as imaging for patients with wounds (i.e. diabetic foot or venous leg ulcers) should be utilized on patients in order to not only check patient compliance with treatments at home, but also to ensure that the most appropriate treatment protocol is being implemented and to ensure that the patients wound is on a healing trajectory.

Commented [KR4]: Do I need to add in anything re RPM? If so what language should be added?

The Alliance offers the following solutions:

Commented [KR5]: Do you agree with these solutions? Are there any others?

1. Reimburse for Use of Digital Tools That Support High-Quality Care

CMS can expand and clarify reimbursement pathways for digital health tools that demonstrably improve outcomes. For example:

- Point of Care Wound Imaging platforms that use AI or 3D technology to assess wound size, depth, and healing trajectory can reduce the need for in-person visits, standardize assessments, and trigger early interventions. Yet, these tools are often underutilized because their use is not explicitly reimbursed. CMS should develop specific billing codes (e.g., for image capture, interpretation, and care planning) for approved wound care imaging tools, especially when used in chronic wound populations like diabetic foot and venous stasis ulcers as well as in pressure injuries.
- RPM platforms can enable continuous monitoring of wound progression, off-loading adherence, or vital signs (e.g., glucose control in diabetic patients), which is especially important for rural patients who face transportation barriers. Expanding the scope of RPM and Chronic Care Management (CCM) billing codes to include more wound-related use cases would incentivize adoption in both rural and urban settings.

2. Support Access and Training for Rural Providers

Rural providers often lack the bandwidth—both technical and operational—to integrate new tools. CMS can help by:

- Providing financial support or grants through CMMI or HRSA to offset upfront costs (e.g., devices, software licenses, connectivity infrastructure).
- Supporting technical assistance and training programs, possibly in partnership with regional extension centers, to educate clinicians and staff on how to integrate imaging and RPM into existing workflows.
- Encouraging partnerships with home health agencies or rural health clinics that can serve as hubs for digital data capture, enabling patients to access imaging tools locally even if their primary provider is remote.

3. Streamline Integration Through Interoperability and Standardization

Providers are more likely to adopt tools that work seamlessly with their existing EHRs and documentation systems. CMS should:

- Promote FHIR-based implementation guides specifically for wound care imaging and RPM data

streams, allowing these tools to feed structured data into clinical documentation and quality reporting frameworks.
- Encourage EHR vendors to offer plug-and-play integration with certified digital health products by tying such capabilities to CEHRT requirements or through public-private partnerships which they are currently not supporting or are charging providers substantive amounts of money to integrate.

4. Link Digital Tool Use to Quality Measures and Value-Based Payment

Finally, CMS should recognize and reward the use of digital health products as part of value-based care delivery:

- For example, providers using wound imaging to document healing progression could meet "Appropriate Use" quality measures, such as documentation of wound stage, off-loading adherence, or infection surveillance.
- Use of RPM to detect early complications could be tied to cost savings metrics and reduced hospitalizations—especially relevant in rural areas where timely in-person care is limited.
- Reward patients for better controlling their blood pressure, glucose, foot pressure or oxygenation at night. Pilot programs that tried this with even modest incentives have been successful.

PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

Barriers in the Development and Deployment of Digital Tools
Significant barriers hinder the development and deployment of digital health tools. These barriers include fragmented data standards, lack of standardized APIs that support both structured and unstructured data, and administrative burdens. CMS can address these challenges by promoting open, standards-based APIs like FHIR, and supporting bulk data access for efficient quality measurement reporting.

Additionally, work-flow saving tools that improve clinical efficiency—such as automated coding and voice-enabled documentation—are often not reimbursed. Despite their proven value, the lack of payment mechanisms discourages adoption. Expanding reimbursement codes for technology-enabled services (e.g., for time saved) could allow for greater use.

Challenges in Quality Measurement and QCDR Integration

In 2014, the US Wound Registry (USWR), in collaboration with the Alliance, developed wound-care-specific quality measures for its Qualified Clinical Data Registry (QCDR). Although these were made freely available and engaged with EHR vendors in order for these measures to be integrated into their EHR systems, the vendors refused to integrate them unless physicians paid large fees. With little incentive to adopt these measures, clinicians instead continued reporting on measures entirely irrelevant to their practice. Even perfect performance often yields bonuses of less than 2%. The result: specialty specific QCDR measures – while well-designed – fail. This failure is not due to the measures being poorly designed, but rather due to barriers of incorporation within the EHR, the high integration costs and the lack of a clear link between quality of care and payment.

Systemic Failures in MIPS and Quality Reporting

The current Merit-based Incentive Payment System (MIPS) is fundamentally flawed. The standard MIPS measures are often too generic or too narrowly focused. Studies have shown MIPS scores are no more effective at distinguishing good from poor care than chance. Evidence also suggests MIPS may have decreased the quality of patient care, particularly for vulnerable populations, and it has dramatically increased physician work and overhead costs.

Furthermore, CMS has historically undervalued process measures—even when these processes (e.g., assessing nutritional status before advanced wound care) are directly linked to improved outcomes. The refusal to recognize such links undermines quality of care. Simple processes around which there is no debate such as off-loading diabetic foot ulcers cannot obtain national MIPS certification because the studies proving the benefit of this vital, basic care are deemed "too small." Essential, evidence-based practices such as off-loading diabetic foot ulcers or compressing venous leg ulcers are excluded for lacking "sufficient" trial size, even when studies with 100+ patients clearly demonstrate their importance and show clear benefit. In fact, relatively small trials in wound care (e.g. 100 patients) that clearly demonstrate the importance of adequate compression to the healing of venous leg ulcers and adequate off-loading of diabetic foot ulcers were deemed by the National Quality Forum as being "too small to be reliable for the development of quality measures." However, in wound care, many of the "process" measures are interventions that lead to healed wounds and are low-cost for which multimillion dollar trials will never be performed.

FHIR and Interoperability: Gaps in Execution

Even when quality measures are built as SMART Apps using the FHIR standard, major EHR vendors frequently block implementation by charging prohibitive fees. While FHIR has potential, it currently lacks the specificity needed for real-world application. A successful system requires focused implementation guides—for example, starting with standardized definitions of vital signs. Without these, data may transmit, but its destination and use remain unclear. Ambiguity like the existence of hundreds of ways to label a "progress note"—renders implementation impractical.

A Flawed Quality Infrastructure

The quality reporting system is deeply perverse. Clinicians are not required to report poor performance, so only the measures with the highest scores are reported. When measure scores are consistently high (inevitable given that only high scores are reported) the measure is considered "topped out" and the measure is retired. Meanwhile, thousands of practitioners cannot or do not perform the needed quality action. Though MVPs (MIPS Value Pathways) offer a more structured measure set, cost measurement methodologies are so inaccurate they are often meaningless. The entire cost reporting framework, in its current form, needs a complete overhaul.

PR-7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem?

The Alliance suggests further leveraging the investment in healthcare information technology (HIT) to implement Medicare payment policy at the point of care, decreasing the cost of expensive auditing programs by increasing the efficiency of human auditors, while ensuring that patients get the treatment they need.

Currently, both prior authorization (used in Medicare Advantage) and post-payment audits (used by Medicare Administrative Contractors for Fee-for-Service patients) are expensive and inconsistent. Review processes vary widely, and auditor checklists often include inaccuracies. These issues have significantly contributed to the backlog of cases awaiting review by Administrative Law Judges.

We believe that HIT presents a better alternative**.** Clinicians could be offered a choice: participate in prior authorization or pre-payment reviews, or instead submit "Appropriate Use" quality measures for high-cost treatments—such as cellular or tissue-based products (CTPs) for wound care. These measures could reflect the core coverage requirements, like diabetic foot ulcer offloading, venous ulcer compression, arterial and nutritional assessments (all of which are processes but required for coverage).
Clinicians would report appropriate use quality measure data to a QCDR as part of a "quality improvement" project under the terms of MIPS. Their performance data could be made public and linked to Medicare payment adjustments if they fall below established, achievable benchmarks. Clinicians with good quality measure performance would be exempt from post payment reviews which can delay payment for years, or prior authorization which often deny needed treatments unfairly.

PR-7: How can the burden of increasing data availability and sharing be mitigated for providers?

Many certified EHRs are unable to meet the requirements of certification. Many are unable to transmit even a simple Continuity of Care Document (CCD) but there are no consequences for them when they are unable to transmit the required data in the required format. Many barriers to data sharing are actually due to poorly designed EHRs that are unable to meet certification specifications.

PR-8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

The Alliance suggests the following:

- Require EHRs to incorporate QCDR measures that are available as SMART apps.
- Reward clinicians who report difficult to pass appropriate use quality measures in lieu of easy to pass, simplistic measures such as reporting BMI and "counseling" patients.

Technology

TD-1: What steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

To stimulate developer interest, CMS should expand APIs to include richer data types (e.g., clinical notes, sensor data), enhance API usability, and reduce data access obstacles. Supporting non-CMS data sources that complement claims data can allow for further innovation.

TD-9: What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality

Revising certification criteria to prioritize comprehensive API-enabled data access and consistent data exchange across all patient record elements would reduce provider burden and improve care coordination.

Value Based Care

VB-1: What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

Value-based care models can drive the adoption of digital health by integrating predictive tools into clinical work flows, supported by aligned incentives and technical standards. In the wound care, such tools can help clinicians assess the likelihood of wound healing and identify the most appropriate products to support optimal outcomes.

AI-assisted Clinical Documentation Integrity tools can streamline chart reviews, identify undocumented diagnoses, and improve the accuracy of diagnosis-related group assignment and risk adjustment, key components of value-based care. However, AI systems must be used with caution, as they can misinterpret clinical nuance, trigger inappropriate compliance queries, or overwhelm providers with unnecessary alerts, increasing burnout. Additionally, reliance on AI without human clinical oversight erodes professional judgment and may introduce unintended bias. CMS should encourage innovation in AI while ensuring transparency, clinical oversight, and enforcing adherence to CDI and query guidelines - ensuring AI enhances rather than undermines ethical, patient-centered, and data driven care.

VB-2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

The Alliance offers the following strategic integration priorities:

1. Artificial Intelligence (AI)

- Leverage AI for decision support, risk coding, and predictive analytics.
- Require transparency in algorithms and encourage CMS-certified AI use in clinical workflows.
- Pilot AI-driven models within the CMS Innovation Center to validate impact.

2. Population Health Analytics

- Mandate analytics platforms to support proactive care and performance tracking.
- Reward organizations integrating social determinants of health and claims-EHR data for full population visibility.

3. Risk Stratification

- Standardize tools for segmenting patients by clinical and social complexity.
- Align stratification outputs with performance benchmarks and shared savings thresholds.

4. Care Coordination

- Tie reimbursement to shared care planning, closed-loop referrals, and telehealth use.
- Promote partnerships with community-based organizations and non-clinical care partners.

5. Usability

- Embed usability requirements into health IT certification for APM participation.
- Encourage use of modular SMART-on-FHIR applications that reduce cognitive load and documentation fatigue.

6. Quality Measurement

- Replace burdensome reporting with automated digital quality measures (dQMs) extracted from structured EHR fields.
- Support AI-assisted quality scoring with provider oversight.

7. Patient Engagement

- Incentivize adoption of patient-facing technologies (e.g., PROs, remote monitoring, secure messaging).
- Integrate shared decision-making tools and longitudinal patient feedback into performance metrics.

Conclusion

The Alliance appreciates the opportunity to provide our feedback on this request for information. We urge CMS to adopt policies fostering reimbursement, interoperability, provider education, and infrastructure support to unlock the full potential of predictive health digital tools. These steps will drive improved patient outcomes, lower wasted costs, increase access to care at home, and facilitate value-based care delivery.

Thank you for your consideration.

Sincerely,

Matthew G. Garoufalis, DPM, FASPS, FACFAOM, CWS
Chair

Kara Couch, MS, CRNP, CWCN-AP, FAAWC
Vice Chair

Marcia Nusgart R.Ph.
Chief Executive Officer

[i] *A list of our members can be found at: www.woundcarestakeholders.org*